

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 79 yrs | Sex: Male | MRN: SMC-04471932
Encounter Date: October 15, 2024 | Provider: Sandra Liu, MD — Medical Oncology | Doc ID: NCI-2024-1015-SURV

Oncology Survivorship Visit

Prostate Cancer — 10-Year Survivorship Surveillance

Subjective

79-year-old, ~11 years from prostate cancer diagnosis and definitive radiotherapy + completed ADT. No urinary obstruction, hematuria, bone pain, or weight loss. Reviewed long-term PSA trend — stable and low. Focus shifting to survivorship and competing comorbidities (HFrEF, CKD).

Long-Term PSA Surveillance

Test	Result	Units	Reference Range	Flag
PSA — Oct 2024 (current)	0.5	ng/mL	Stable, sub-threshold	
PSA — 2021	0.9	ng/mL	Prior	
PSA — 2019 (bounce peak)	1.4	ng/mL	Prior (benign bounce)	
PSA — 2015 (nadir)	0.2	ng/mL	Nadir	
Testosterone, total	388	ng/dL	264 - 916	

Assessment

Prostate cancer in long-term remission. The 2019 PSA rise to 1.4 followed by decline to 0.5 confirms a benign post-radiation PSA bounce rather than recurrence (never met Phoenix threshold of nadir +2). No evidence of biochemical or clinical recurrence. Late radiation effects: none significant. Survivorship issues: bone health (osteoporosis on denosumab), cardiometabolic risk.

Plan

- De-escalate to annual PSA given >10-year stability and competing comorbidity; symptom-prompted evaluation otherwise.
- Continue bone-health therapy (prior ADT contributor).
- No oncologic treatment required. Coordinate overall care with PCP/cardiology/nephrology.
- Survivorship care plan provided; reassurance given.